

Dapsone

Dapsone is a valid therapeutic option for dermatitis herpetiformis during the 1- to 2-year period required for a gluten-free diet (GFD) to become fully effective. A dosage of 1 mg/kg/day can control itching and blister formation. Although dapsone effectively reduces pruritus and inflammatory skin lesions, patients must be closely monitored—particularly for renal and liver function—due to the risk of severe adverse effects. The most common side effect is hemolysis, and patients should be evaluated within 2 weeks of starting therapy, as hemolysis can be acute in some individuals.

Adverse reactions to dapsone are classified as either toxic or idiosyncratic and are typically dose-dependent. They are more frequent in patients with comorbidities such as anemia, cardiopulmonary disease, or glucose-6-phosphate-dehydrogenase (G6PD) deficiency. Dapsone can also induce a severe hypersensitivity syndrome in approximately 5% of dermatitis herpetiformis patients, occurring 2–6 weeks after treatment initiation. This syndrome includes fever, rash, malaise, lymphadenopathy, and variable visceral involvement.

Table 4: Main adverse reactions to dapsone

- **Toxic reactions:**
 - Hemolysis
 - Methemoglobinemia
 - Dose-dependent hepatotoxicity
 - Peripheral neuropathy
- **Idiosyncratic reactions:**
 - Hypersensitivity syndrome (DRESS)
 - Agranulocytosis
 - Hepatitis
 - Skin rashes

Table 5: Indications for monitoring

- GFD adherence
- Possible development of autoimmune associated diseases (despite GFD adherence)
- Metabolic alterations (dyslipidemia, non-alcoholic steatohepatitis)
- Possible development of neoplastic (e.g., lymphoma) or non-neoplastic complications (e.g., refractory celiac disease, ulcerative jejunoileitis, collagenous sprue)

Table 6: Item list for routine check-ups

- Physical examination
- Dietician counselling
- Serological laboratory tests:
- Hemochrome
- Malabsorption evaluation (sideremia, ferritin levels, mean corpuscular volume, glucose, thyroid hormones)
- Immunological and autoimmune markers (dermatitis herpetiformis-specific and non-specific autoantibodies)

Sulfasalazine and Sulphamethoxypyridazine

Sulfasalazine and sulphamethoxypyridazine may serve as effective alternatives to dapsons, particularly when dapsons is ineffective or associated with adverse events. The recommended dosages are 1–2 g/day for sulfasalazine and 0.25–1.5 g/day for sulphamethoxypyridazine.

Both drugs share similar adverse effects, including hypersensitivity reactions, hemolytic anemia, proteinuria, and crystalluria. Therefore, a complete blood count with differential and urine microscopy with urinalysis should be

performed before treatment initiation and monthly for the first 3 months, then every 6 months thereafter. The most common adverse effects—nausea, anorexia, and vomiting—can often be minimized by using enteric-coated formulations.

Corticosteroids

Oral corticosteroids are generally ineffective in dermatitis herpetiformis. However, topical application of potent or very potent corticosteroids, such as clobetasol propionate, can help reduce pruritus and skin inflammation.

Antihistamines

Although antihistamines have limited efficacy in dermatitis herpetiformis, third-generation agents with specific activity on eosinophilic granulocytes may be considered a third-line treatment option for symptom control, particularly for pruritus and itching.

Dermatitis

Herpetiformis

Monitoring

Regular follow-up is essential to confirm diagnosis through objective response to a gluten-free diet and to identify non-compliance. Patients with dermatitis herpetiformis and associated celiac disease should be evaluated at regular intervals—initially 6 months after diagnosis and then annually—by a multidisciplinary healthcare team including a physician and a dietician.